

Erosive and Ulcerative Lesions in Oral Lichen Planus

Erosive to ulcerative lesions are more frequently observed in severe cases of oral lichen planus. These lesions commonly affect the buccal mucosa, alveolar epithelium, and gingival sulci, and may extend posteriorly to involve the pharynx and laryngeal region. Although rare, erosive involvement of the esophagus can lead to dysphagia and esophageal stenosis.

Follicular Lichen Planus

Follicular lichen planus may occur in isolation or alongside other cutaneous or mucosal forms of lichen planus. This variant is also known as **lichen planopilaris**, with alternative terms including lichen planus follicularis, peripilaris, and acuminatus. It presents as keratotic follicular papules and studded plaques, primarily on the trunk and medial aspects of the proximal extremities.

When the scalp is affected, follicular lichen planus can result in **cicatricial alopecia** (see *Lichen Planus of the Scalp; Fig. 26-9*). A distinct clinical triad—follicular lichen planus of the skin (lichen planus spinulosus) and/or scalp, multifocal cicatricial alopecia of the scalp, and nonscarring alopecia of the axillary and pubic regions—is recognized as **Graham-Little-Piccardi-Lassueur syndrome** (also known as Graham-Little-Feldman syndrome). In some cases, more typical skin and nail lesions of lichen planus may also be present.

Other variants of follicular lichen planus include: - **Pseudopelade of Brocq** – characterized by patchy, atrophic scarring alopecia - **Lichen planus follicularis tumidus** – presenting with oval pseudotumoral plaques in the mastoid region - **Frontal fibrosing alopecia** – a post-menopausal variant causing progressive frontal hairline recession - **Lichen planopilitis** – a rare form with lichenoid inflammation centered on the acrosyringium and eccrine ducts entering the epidermis

Lichen Planus Pigmentosus

Lichen planus pigmentosus is marked by the presence of hyperpigmented, dark-brown macules, typically appearing in sun-exposed areas and flexural folds. It is more commonly seen in individuals with darker skin tones, particularly Latin Americans and those of Asian or Middle Eastern descent.

Histologically, the condition shows: - Atrophic epidermis - Vacuolar alteration of the basal cell layer - Sparse lymphohistiocytic lichenoid infiltrate - Pigment incontinence

This variant closely resembles **ashy dermatosis** (erythema dyschromicum perstans), and the two may represent overlapping manifestations within the spectrum of lichenoid inflammation in pigmented skin. Ethnic and genetic factors likely influence disease expression (see Chap. 73). While histopathologic features are similar, some investigators emphasize distinct clinical, histologic, and immunopathologic differences among lichen planus pigmentosus, ashy dermatosis, and related disorders.